

D117

Advanced Health Assessment for the Advanced Practice Nurse

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

Method © 2026 Erik Daniel Lindquist — Course Mastery Coach, armored drill. Items drafted with AI assistance under his direction.

How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 36-year-old software engineer who enjoys rock climbing reports three days of sore throat. The clinician documents temperature 38.6 °C, tonsillar exudate, and tender anterior neck nodes. A rapid strep test is positive. Where does the positive rapid strep result belong?

- A. Plan.
- B. Objective.
- C. Assessment, since it confirms the diagnosis.
- D. Subjective.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A clinician writes: “Streptococcal pharyngitis; rule out peritonsillar abscess.” Which section is this?

- A. Objective.
- B. Assessment.
- C. Plan.
- D. Subjective.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A nurse practitioner at a clinic with a new waiting-room aquarium examines the abdomen of a patient with cramping. The patient ate a large lunch. After inspection, what comes next?

- A. Auscultation.
- B. Percussion.
- C. Wait for digestion before any exam.
- D. Palpation.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A 64-year-old retired bus driver has calf pain after a long flight. He has no calf swelling, no redness, and no shortness of breath. He loves crossword puzzles. Why record the absent swelling and dyspnea?

- A. They are pertinent negatives that help weigh DVT and pulmonary embolism.
- B. They matter mainly for his hobbies.
- C. They prove he has no clot.
- D. Only abnormal findings need recording.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A 55-year-old warehouse manager who coaches Little League has smoked 1.5 packs a day for 20 years. He switched to low-tar cigarettes last year. How many pack-years is his history?

- A. 20 pack-years
- B. 30 pack-years
- C. 21.5 pack-years
- D. 600 pack-years

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A patient has a 40 pack-year history from smoking 2 packs a day. How many years has she smoked?

- A. 80 years
- B. 40 years
- C. 38 years
- D. 20 years

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A 45-year-old teacher who bikes to work weighs 90 kg and is 1.75 m tall. Her blood pressure is 126/78. What is her BMI category?

- A. Normal, because she bikes to work.
- B. Obese (BMI about 51.4).
- C. Obese, since any BMI over 25 is obese.
- D. Overweight (BMI about 29.4).

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A patient 1.60 m tall has a BMI of exactly 30. What does she weigh?

- A.** 18.75 kg
- B.** 48 kg
- C.** 76.8 kg
- D.** 11.7 kg

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

9.

A 74-year-old retired chef with long-standing hypertension and left ventricular hypertrophy has an extra sound heard just before S1 at the apex. He also takes a statin. What does this sound suggest?

- A.** A normal finding in all older adults.
- B.** A split S2.
- C.** An S4 from a stiff, hypertrophied left ventricle.
- D.** An S3 from volume overload.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

10. [TRANSFER — reversed stem / different variable]

A 70-year-old with heart failure has an extra sound heard just after S2. What is it most likely?

- A.** S3, suggesting volume overload.
- B.** A normal finding at any age.
- C.** An ejection click.
- D.** S4.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

11.

A 3-year-old at a daycare with a new playground has a high-pitched sound on inspiration heard without a stethoscope, drooling, and distress. Her older brother has asthma. What is this sound, and what does it suggest?

- A.** A pleural friction rub.
- B.** Fine crackles from fluid.
- C.** Stridor, suggesting upper-airway obstruction needing urgent care.
- D.** Wheezing from asthma, like her brother.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

12.

A 38-year-old with a long history of migraines, who works as a graphic designer, reports a sudden headache that reached maximum intensity within one minute, calling it the worst of her life. Her usual migraine medicine is in her purse. What is the best next step?

- A.** Diagnose migraine, since she has a history.
- B.** Schedule a routine follow-up in two weeks.
- C.** Have her take her usual migraine medicine.
- D.** Arrange urgent evaluation for subarachnoid hemorrhage.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

A 36-year-old software engineer ~~who enjoys rock climbing~~ reports three days of sore throat. The clinician documents temperature 38.6 °C, tonsillar exudate, and tender anterior neck nodes. A rapid strep test is positive. Where does the positive rapid strep result belong?

Target word: SOAP section **Decides:** A test result the clinician obtained.

A	Adjacent category	Plan lists what will be done next.
B	Correct	Test results are objective data.
C	Confusion set	The diagnosis goes in A; the result itself is O.
D	Inverted operation	Subjective is what the patient reports.

2. Answer B Concept 1 • transfer

A clinician writes: “Streptococcal pharyngitis; rule out peritonsillar abscess.” Which section is this?

Target word: SOAP section **Decides:** Diagnosis and differential.

A	Inverted operation	O holds findings, not conclusions.
B	Correct	The clinician's diagnostic judgment goes in A.
C	Confusion set	Plan is treatment, tests, and follow-up.
D	Adjacent category	S is the patient's report.

3. Answer A Concept 2

A nurse practitioner ~~at a clinic with a new waiting room aquarium~~ examines the abdomen of a patient with cramping. ~~The patient ate a large lunch.~~ After inspection, what comes next?

Target word: next step **Decides:** Auscultate before touching.

A	Correct	Listening before touch avoids altering bowel sounds.
B	Dropped element	Skips auscultation.
C	Camouflage	The lunch does not change the sequence.
D	Inverted operation	Palpation comes last in the abdomen.

4. Answer A Concept 3

A 64-year-old retired bus driver has calf pain after a long flight. He has no calf swelling, no redness, and no shortness of breath. ~~He loves crossword puzzles.~~ Why record the absent swelling and dyspnea?

Target word: value of absent findings **Decides:** Pertinent negatives shape the differential for DVT/PE.

A	Correct	Absent key findings refine the differential.
B	Camouflage	Unrelated to the clinical question.
C	Absolute language	Negatives lower but do not eliminate probability.
D	Inverted operation	Leaves out data that shape the differential.

5. Answer B Concept 4

A 55-year-old warehouse manager ~~who coaches Little League~~ has smoked 1.5 packs a day for 20 years. ~~He switched to low tar cigarettes last year.~~ How many pack-years is his history?

Target word: pack-years **Decides:** 1.5×20 .

A	Dropped element	Uses years only.
B	Correct	$1.5 \times 20 = 30$.
C	Inverted operation	Adds packs and years.
D	Wrong stem number	Uses cigarettes per day (30) times years.

6. Answer D Concept 4 • transfer

A patient has a 40 pack-year history from smoking 2 packs a day. How many years has she smoked?

Target word: solve for years **Decides:** $40 / 2$.

A	Inverted operation	Multiplies instead of divides.
B	Wrong stem number	The pack-year total, not years.
C	Confusion set	Subtracts packs from pack-years.
D	Correct	$40 / 2 = 20$.

7. Answer D Concept 5

A 45-year-old teacher ~~who bikes to work~~ weighs 90 kg and is 1.75 m tall. ~~Her blood pressure is 126/78.~~ What is her BMI category?

Target word: BMI category **Decides:** $90 / 1.75^2$.

A	Camouflage	Activity does not change the calculation.
B	Dropped element	Divides by height, not height squared ($90 / 1.75$).
C	Absolute language	25–29.9 is overweight; obese starts at 30.
D	Correct	$90 / 3.0625 = 29.4$.

8. Answer C Concept 5 • transfer

A patient 1.60 m tall has a BMI of exactly 30. What does she weigh?

Target word: solve for weight **Decides:** $\text{BMI} \times \text{height}^2$.

A	Inverted operation	Divides BMI by height.
B	Dropped element	Multiplies by height, not height squared.
C	Correct	$30 \times 2.56 = 76.8$.
D	Confusion set	Divides BMI by height squared.

9. Answer C Concept 6

A 74-year-old retired chef with long-standing hypertension and left ventricular hypertrophy has an extra sound heard just before S1 at the apex. ~~He also takes a statin.~~ What does this sound suggest?

Target word: meaning of the sound **Decides:** Late diastolic sound before S1.

A	Absolute language	S4 is common but signals reduced compliance.
B	Confusion set	Split S2 occurs at the end of systole, not before S1.
C	Correct	S4 reflects atrial kick into a noncompliant ventricle.
D	Inverted operation	S3 comes early in diastole, just after S2.

10. Answer A Concept 6 • transfer

A 70-year-old with heart failure has an extra sound heard just after S2. What is it most likely?

Target word: sound just after S2 **Decides:** Early diastolic sound.

A	Correct	Early diastolic filling into a failing ventricle.
B	Absolute language	S3 in older adults is abnormal.
C	Adjacent category	Clicks occur early in systole.
D	Inverted operation	S4 comes just before S1.

11. Answer C Concept 7

A 3-year-old ~~at a daycare with a new playground~~ has a high-pitched sound on inspiration heard without a stethoscope, drooling, and distress. ~~Her older brother has asthma.~~ What is this sound, and what does it suggest?

Target word: name the sound **Decides:** High-pitched inspiratory upper-airway sound.

A	Adjacent category	Rubs are grating sounds with breathing, not high-pitched inspiratory sounds.
B	Confusion set	Crackles are discontinuous and not heard this way.
C	Correct	Inspiratory stridor signals upper-airway compromise.
D	Camouflage	Family history does not make this wheezing.

12. Answer D Concept 8

A 38-year-old with a long history of migraines, ~~who works as a graphic designer,~~ reports a sudden headache that reached maximum intensity within one minute, calling it the worst of her life. ~~Her usual migraine medicine is in her purse.~~ What is the best next step?

Target word: next step **Decides:** Thunderclap onset is a red flag.

A	Adjacent category	A new pattern needs its own evaluation.
B	Inverted operation	Delays care for a red flag.
C	Camouflage	Anchors on history and the available medicine.
D	Correct	Sudden worst-ever headache needs emergency workup.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	5	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	10	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	6	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	SOAP note placement	S = what the patient reports; O = exam findings, vitals, and test results; A = the clinician's diagnosis or differential; P = tests, treatment, teaching, and follow-up.	Putting a vital sign in S or a diagnosis in O.	"Where in the note does this belong?"
2	Abdominal exam sequence	Inspect, auscultate, percuss, then palpate, so touch does not change bowel sounds.	Using the head-to-toe inspect-palpate-percuss-auscultate order for the abdomen.	An abdominal exam step order question.
3	Pertinent positives and negatives	Pertinent negatives are absent findings that help rule a diagnosis in or out; they belong in the record because they shape the differential.	Recording only what is abnormal.	A key symptom or sign that is absent.
4	Pack-years	Pack-years = packs smoked per day × years smoked.	Adding packs and years, or counting cigarettes instead of packs.	Daily smoking amount and years smoked.
5	Body mass index	BMI = weight in kg / (height in m) ² ; 18.5–24.9 normal, 25–29.9 overweight, 30 or above obese.	Dividing by height rather than height squared.	Weight in kg and height in meters.
6	S3 vs. S4	S3 is early diastolic and suggests volume overload or heart failure in adults (can be normal in the young); S4 is late diastolic, just before S1, and suggests a stiff ventricle.	Mixing up which sound comes just before S1.	Timing relative to S1 and S2, with history of heart failure or hypertension.
7	Adventitious lung sounds	Crackles suggest fluid or opening of collapsed small airways; wheezes suggest narrowed airways; stridor is a high-pitched inspiratory upper-airway sound that is an emergency sign.	Calling stridor a wheeze and treating it as routine.	Pitch, phase, and location of an added sound.
8	Red flags change the workup	Red-flag features (such as a sudden worst-ever headache) call for urgent evaluation even when a benign cause is common.	Anchoring on a patient's history of a benign condition.	A familiar complaint with a new, abrupt, or severe feature.

Course scope (public description): Advanced Health Assessment prepares the graduate nursing student to perform the role of an advanced practice nurse with the essential competencies necessary to provide primary health care to patients and families of diverse populations. Students will develop the skills needed for systematically collecting and analyzing subjective and objective patient data. Through simulation and clinical experiences, students will use data to determine current and ongoing patient health status, predict health risks, and identify health-promoting activities for patients across the lifespan. Advanced Health Assessment will prepare the nursing graduate with the critical thinking, clinical reasoning, and advanced diagnostic skills required for advanced practice nursing. Upon completion of Advanced Health Assessment, the graduate will be able to synthesize individual and systems level subjective and objective data to facilitate the differential diagnosis processes. Also, the graduate will be able to clearly describe to patients and providers the pertinent health

assessment findings and rationale supporting the diagnostic process.